



Armada Kalamunda Group

Spontaneous Labour and Birth Guideline

1. Purpose

This guideline focuses on the management of spontaneous labour in healthy women at term, with a singleton vertex fetus, with no maternal or fetal risk factors. Labour management should support the natural physiological process of birth, while identifying potential concerns. The purpose of this guideline is to provide guidance for the intrapartum management of normal labour, to increase the likelihood of a vaginal birth, and to optimise maternal and fetal outcomes.

2. Guidelines

The spontaneous vaginal birth of an infant is a physiological event and can be facilitated by a midwife, student midwife (under the midwife's supervision) and medical practitioner. Women are admitted as a public patient under the GP Obstetrician on call for the day or their nominated GP if they are a GP Own (public), or as a private patient. That practitioner should review the woman as required. Care in labour will be conducted by the midwife who is to notify the medical practitioner of any concerns arising during the normal labour and birth process.

When indicated:

- There will be an increase in the frequency of recommended observations as required.
- Care will be modified in line with relevant obstetric guidelines to individual circumstances while maintaining a focus on supporting the principles of normal labour and birth.

3. First stage of labour

There is good evidence that guidelines aimed at discouraging early admissions and delaying childbearing women's hospital admission until the active phase leads to a positive increase in normal labour and birth rates, with a consequent reduction in intrapartum interventions and caesarean section rates.

One of the key recommendations in the WHO guideline on intra-partum care for a positive childbirth experience recognizes that every labour is unique and that women do not all progress at the benchmark rate of 1 cm/hour of cervical dilatation (WHO, 2018).

There are two identified phases of the first stage of labour:

- Latent phase
- Active phase

3.1 Latent phase of labour

The latent labour phase of first stage is a period characterized by painful uterine contractions and variable changes of the cervix, including some degree of effacement and progression of dilatation to less than 5 cm. Women should be informed that the duration of latent phase is difficult to measure and can vary widely from one woman to another.

3.2 Labour – telephone triage

During the latent phase of labour, the woman may contact the Antenatal Assessment Unit or their care provider requesting support and advice. An early assessment of labour is provided through telephone triage by the hospital Antenatal Assessment Unit (AAU) midwives as per the [Antenatal Clinical Guideline](#).

The AAU midwife will document the telephone assessment, advice provided and the agreed plan on a Telephone Advice form (AKMR9).

3.3 Labour – initial assessment and diagnosis

On presenting to AAU in the latent phase of labour a Maternal Assessment ≥ 20 weeks (AKMR8) form is commenced, and an initial maternal and fetal wellbeing assessment will be completed by the midwives. A comprehensive midwifery assessment will be conducted which will include:

- Obtaining a history of the presenting concern
- Review WA Hand Held Record
- A baseline set of maternal observations including vaginal loss.
- An abdominal palpation and assessment of contraction history, strength, and frequency
- A fetal heart rate (FHR) and fetal movement pattern
- A vaginal examination and or speculum examination where indicated.

If the active phase of labour is diagnosed, then admit for one-to-one labour and birth care.

Refer to Appendix A: [Initial Maternal and Fetal Assessment](#)

3.4 Determined in Latent phase of labour – Not requiring Birth Suite

Women presenting in the latent phase of labour may be experiencing painful contractions without significant cervical change. It is not uncommon for labour to stop and start multiple times before labour is finally established. The Midwife will

- Conduct complete antenatal assessment as outlined above.
- Offer her individualised support, reassurance and analgesia if needed. Patient must be admitted if they receive narcotics.
- Offer to return to place of residence if within vicinity of Armadale Health Service catchment area. The woman must be reassured that she can call or return to the hospital at any time. Advise the woman to contact AAU if:
 - Her condition changes or concerns arise such as vaginal bleeding.
 - Spontaneous Rupture of Membranes
 - Changes in vaginal loss such as green/yellow discharge or fluid
 - There is a change or decrease in fetal movements.
 - Increased frequency and strength of contractions
 - Constant pain or
 - Any concern that requires attention and assessment

3.5 Latent phase of labour – Prolonged

Whilst there is limited evidence to provide a definition, where the latent phase is considered prolonged and/or the woman is requiring stronger analgesia, the GP Obstetrician or Specialist Obstetrician should be consulted. An agreed plan should be made with the woman as to whether she should return to AAU for assessment, can remain at home with a plan to return, be admitted to hospital, or whether an induction of labour is commenced.

N.B. Women contacting AAU for the 3rd time in latent phase, must be asked to attend AAU to be assessed and to discuss and develop an ongoing plan.

CALD women who present twice in the latent phase, should be referred to the GP Obstetrician and augmentation considered.

3.6 Active phase of labour

The onset of active labour is recognised as the time point from 4- 5cm dilation associated with cervical changes, in the presence of regular painful contractions it is recommended that the woman be admitted for one-to-one labour and birth care. Active first stage is completed at full cervical dilation. The midwife will:

- Ensure labour room equipment is prepared and ready to receive the woman and neonate.
- Ensure there is a 3-point identification check and apply an identification label to a limb of the woman. Advise clerical staff of the admission to birth suite.
- Notify the medical practitioner of admission and findings of initial assessment using an ISOBAR handover format.

Refer to Appendix D: [First Stage of Labour](#)

In active labour dilatation of 0.5 cm per hour is generally the limit of what is considered normal progress. Purwar et al (2021) All aspects of progress should be considered:

- Maternal and fetal condition
- Cervical dilatation and rate of change
- Descent and rotation of the fetal head
- Strength, duration, and frequency of contractions
- Parity
- Previous labour history
- Slowing of progress in the multiparous woman
- Use of analgesia

3.7 Maternal observations in active labour

Consideration must be given to the woman's medical and obstetric history and present clinical condition when determining the frequency of observations. Increase the frequency of observations if clinically indicated and escalate as per the Antenatal Maternity Adult Deterioration Detection System Observation and Response Chart (AKMR140.8)

Refer to Appendix B: [Observations in First and Second Stage](#)

3.8 Fetal observations

During labour with no identified risk factors, intermittent auscultation (IA) of the fetal heart rate (FHR) with a handheld fetal heart rate monitoring doppler must occur before and during a contraction and continue for at least 1 minute after the contraction has ceased.

- IA will be documented every 15-30 minutes on Intellispace Production Program (ISP) - Notes.
- The Maternal Pulse should be palpated and documented to differentiate between maternal and fetal heart rates. Document this on the partogram and on ISP.
- The midwife will document on ISP that IA is being used to auscultate the FHR

If risk factors are identified as per [EMHS Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy EMHS: 195](#) a CTG will be commenced.

3.9 Documentation

- Partogram must be completed using the Intellispace Production Program if using CTG monitoring.
- If using Intermittent Auscultation, FHR is documented on the Partogram (AKMR 72).
- All documentation during labour including progress notes and fresh eyes reviews, must be documented on the Intellispace Production Program.
- Maternal Observations during labour will be documented on the Antenatal MADDS Chart AKMR 140.8
- If an IT outage occurs all documentation will revert to paper based.

4. Second stage of labour

The duration of second stage of labour should be determined by clinical judgement which includes analgesia use, maternal and fetal condition, and progress of the presenting part through the pelvis.

Refer to Appendix E: [Second Stage of Labour](#)

Ensure the Labour ward co-ordinator remains aware of the woman's progress

4.1 Assessment of labour progress

- Allow passive descent of the presenting part over 1 hour.
- If the woman has no urge to push after 1 hour from confirmed full dilation this must be discussed with the Labour Ward Coordinator and GP Obstetrician.
- Observe for signs of advancement of presenting part.

4.2 Maternal observations in second stage of labour

Consideration should be given to the woman's medical and obstetric history and present condition when determining the frequency of observations.

Refer to Appendix B: [Fetal and Maternal Observations in First and Second Stage](#)

4.3 Fetal observations

- CTG monitoring to continue during second stage if commenced during the first stage of labour. It is to be commenced if any concerns or if there is a delay in the second stage.
- Intermittent Auscultation of the FHR using a handheld fetal heart rate doppler must occur at least 5 minutely and/or before, during and after each contraction. Maternal pulse must be palpated and documented concurrently with the FHR and be documented on the Partogram (AKMR 72).
- A CTG will be commenced if intermittent auscultation is abnormal or there is a delay in labour progress.
- If fetal observations are unable to be recorded as above the reason must be clearly documented and the Labour Ward Coordinator must be informed.
- ([EMHS Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy EMHS: 195](#))

Refer to Appendix B: [Fetal and Maternal Observations in First and Second Stage](#)

4.4 Guidance on the duration of second stage

Delay is defined by NICE (2014) as an active second stage lasting over 2 hours for nulliparous and 1 hour for multiparous women. It is important to diagnose delay in second stage and seek obstetric opinion to ensure timely delivery.

- Time limitation for duration of second stage should be determined by clinical assessment of the:
 - Rotation and/or advancement and descent of the presenting part
 - Maternal and fetal wellbeing
- If there is no significant urge to push, with or without regional anaesthesia, the woman can be given an hour of passive second stage to allow descent and/or rotation of the presenting part.
- In the presence of regional anaesthesia, a delay in second stage should be defined as >3 hours in a nullipara and >2 hours in a multipara.
- The use of warm compresses to the perineum is recommended in second stage to reduce the incidence of perineal trauma.
- A CTG will be performed when a diagnosis of delay in second stage is made and half-hourly documentation should be made regarding: maternal effort, progress and the strength and frequency of contractions.
- When a diagnosis of delay is made an obstetrician will be informed and an individualised management plan for delivery or further review will be made.

5. Birth

The midwife is responsible for the checking and preparation of the area including:

- Checking PANDA resuscitation cot ensuring equipment is stocked and temperature is set to warm.
- Delivery Trolley is available and prepared for birth including a sterile birth pack, episiotomy scissors and abdominal sponges.
- Staff are wearing personal protective equipment including a waterproof apron and safety glasses.

- Oxytocin has been discussed with the patient and if consented for active management third stage, the Oxytocin is available and appropriately labelled.
 - For Low-Risk women administer Oxytocin 10 units IM
 - For High-Risk women provided there are no pre-existing contraindications, administer one ampoule of Syntometrine (Oxytocin 5 units and Ergometrine 0.5mg) IM.

The Midwife role:

- Determine the requirement to request a Gp Obstetrician and/or Paediatrician or Paediatric Register to be present at the birth.
- Understand that episiotomy is not routinely performed, but in selected cases it may reduce the incidence of 3rd and 4th degree tears – Obstetric Anal Sphincter Injuries (OASIS). Discuss with all women prior to the birth about episiotomy and obtain informed verbal consent prior to performing an episiotomy. Discussion in relation to performing an episiotomy and information provided to the woman must be documented in ISP.
- Apply perineal support for controlled delivery of the head.
- Document the time of the delivery of the head and time of birth of baby on ISP and the Partogram (AKMR 72)
- Administer oxytocic preferably within one minute after the birth of the baby with the woman's informed consent. Consent to be documented in ISP.
- Delay clamping of the umbilical cord for a minimum of 2 minutes following birth unless neonate requiring medical intervention to support transition to extrauterine life.
- Clamp the umbilical cord 1-2 centimetres from the umbilicus.
- Monitor the newborn for transition to extrauterine life and escalate appropriately.
- Collect cord blood for pathology for neonates born to Rh(-) mothers and mothers who are blood group O Positive.

6. Infant care

- Assess Apgar score at one and five minutes of life on the neonate.
- Initiate neonatal resuscitation as required.
- Obtain the parents consent prior to performing any procedures or observations on the neonate. Document consent in the Neonatal DMR record.
- Apply the SaO₂ probe to the neonates right wrist for routine SaO₂ monitoring
- Maintain skin to skin with mother and neonate for a minimum of 1 hour post birth or until neonate successfully breastfed.
- Encourage early breastfeeding within the first hours of life by supporting skin to skin contact and allowing the baby to root and latch spontaneously.
- Label neonate with x 2 ID tags cross checked by the mother.
- Document the birth stating the gender, date of birth, time of birth, gestation at birth, condition of the baby at birth, Apgar scores, cephalocaudal check, if the baby passed meconium or urine, baby weight, height and length in the Neonatal DMR record. Including any risk factors and plan of care for the neonate

Refer to:

- [Neonatal Routine and Additional Observations PAE-GUI-0288-16](#)

7. Third stage of labour

- Note time of delivery of placenta and membranes

Refer to either:

- [Birth - third stage of labour - active management OBS-GUI-0054-11](#)
- [Birth - third stage of labour - Physiological / expectant management OBS-GUI-0055-11](#)

8. Immediate and ongoing care following labour and birth (4th stage)

- Inspect the perineum for perineal trauma with the woman's informed consent. If suturing is deemed as necessary, refer to the GP Obstetrician or credentialed midwife for further assessment and suturing.
- Monitor fundal height and document findings.
- Monitor vaginal blood loss, escalate if concerns that vaginal blood loss is moderate to heavy. [Postpartum Haemorrhage \(PPH\) Guideline](#)
- Assess blood loss, either estimate or measure and document findings on the Partogram (AKMR 72) and in DMR.
- To prevent inadvertent equipment retention, all swabs and equipment must be counted and verified with another person when a delivery pack or perineal pack is opened and at completion of use. This information should be documented and both checkers must sign as correct – see AKMR72 and AKMR72.1
- Maternal blood collected if RH negative blood group and sent to pathology. Refer to [Rhesus Negative Women Guideline](#)

8.1 Maternal observations following labour and birth

- Complete maternal observations on Partogram (AKMR 72) every 30 minutes following completion of third stage of labour until transfer to Postnatal Ward.
- Prior to transfer to the postnatal ward document maternal observations including BP, heart rate, temperature, respiration rate, Sao², AVPU score, fundal height and vaginal loss on the Postnatal - Maternity Adult Deterioration Detection System Observation and Response Chart (MADDS AKMR140.9)
- Monitor perineum for haemostasis, union and swelling.
- Bladder care – encourage early voiding, assess emptying of bladder and sensation to void as per [Postpartum Urinary Retention Guideline](#)
- Prior to ambulation perform the Bromage score, women must have a Bromage score of 1 prior to ambulation.

8.2 Disposal of placenta and membranes

- On completion of checking, place in a plastic bag and container with a firm fitting lid label the container with woman's MRN number, date and time of birth.
- Place container into a plastic bag. Place the container in placenta refrigerator in the birth suite pan room.
- If the woman wishes to take it home, ensure she complete the Release of Human Tissue and explained Medical Device Consent AKMR 30.3 and refer to: [Birth - Women Taking Placenta Home](#)

8.3 Documentation

- Complete Documentation of the birth
 - AHS Partogram AKMR72
 - DMR
 - Enter birth details in the Birth Register
 - Complete Stork birth entry. Print the “Labour and birth summary” from STORK and sign before placing in patients file.
 - Complete appropriate risk assessment tools, in the maternity care plan
 - Complete Centrelink paperwork and cot card
 - Commence Child Health Book
 - Commence AHS Maternity Care Plan
 - Commence AHS Neonatal History and assessment AKMR78.1

8.4 Prior to ward transfer

- Commence the Maternity Adult Deterioration Detection System Observation and Response Chart (MADDS AKMR140.8)
- Complete the Clinical Handover Birth Suite to Postnatal Ward Mother in Maternal Care plan.
- Complete the Clinical Handover Birth Suite to Postnatal Ward in Neonatal Care Plan
- Update Clinical Manager (iCM) Handover sheet.
- Discuss transfer with birth suite coordinator who will confirm bed availability and staff able to accept care with the ward coordinator.
- Transfer the woman and her infant, with infant in cot to the ward by either wheelchair or walking dependent on the woman’s condition.

8.5 Arrival to postnatal ward

- Undertake comprehensive bedside clinical handover using ISOBAR format with Midwife or Registered Nurse receiving patient care.
- Check baby’s identification bands are correct with the receiving midwife.

9. Legislative context

The following documents are required to give effect to this policy:

- Health Department of Western Australia (HDWA) [Clinical Handover Policy MP 0095](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)
- [EMHS Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy EMHS: 195](#)

10. Supporting information

The following documents support the implementation of this policy:

- [AHS Post-partum Urinary Retention Guideline OBS-PRO-0366-15.](#)
- [Birth - Third Stage of Labour - Active Management OBS-GUI-0054-11](#)
- [Birth - Third stage of Labour - Physiological / Expectant Management OBS-GUI-0055-11](#)
- [Recognition and Response to Acute Deterioration AKG-POL-0388-14](#)

- [Birth – Women Taking Placenta Home OBS-GUI-0056-15](#)

11. Compliance, monitoring and evaluation

Compliance against this guideline will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS

Specific findings are to be reported at the Obstetrics Safety and Quality Departmental meeting for review and identification of any opportunities for improvement.

12. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 6 – Action 6.3
- Standard 6 – Action 6.5
- Standard 6 – Action 6.7
- Standard 6 – Action 6.9
- Standard 8 – Action 8.6
- Standard 9 – Action 8.11

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14. Document owner

Head of Obstetrics and Gynaecology

Enquiries relating to this guideline may be directed to:

Title: Midwifery Manager
 Department: Obstetrics and Gynaecology
 Email: Shannon.scari@health.wa.gov.au

15. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3.0	17/12/2024	17/12/2027	Scheduled review major changes – refer to obstetric team for further clarification.

16. Authorisation

This guideline has been authorised and issued by the Director of Clinical Services.

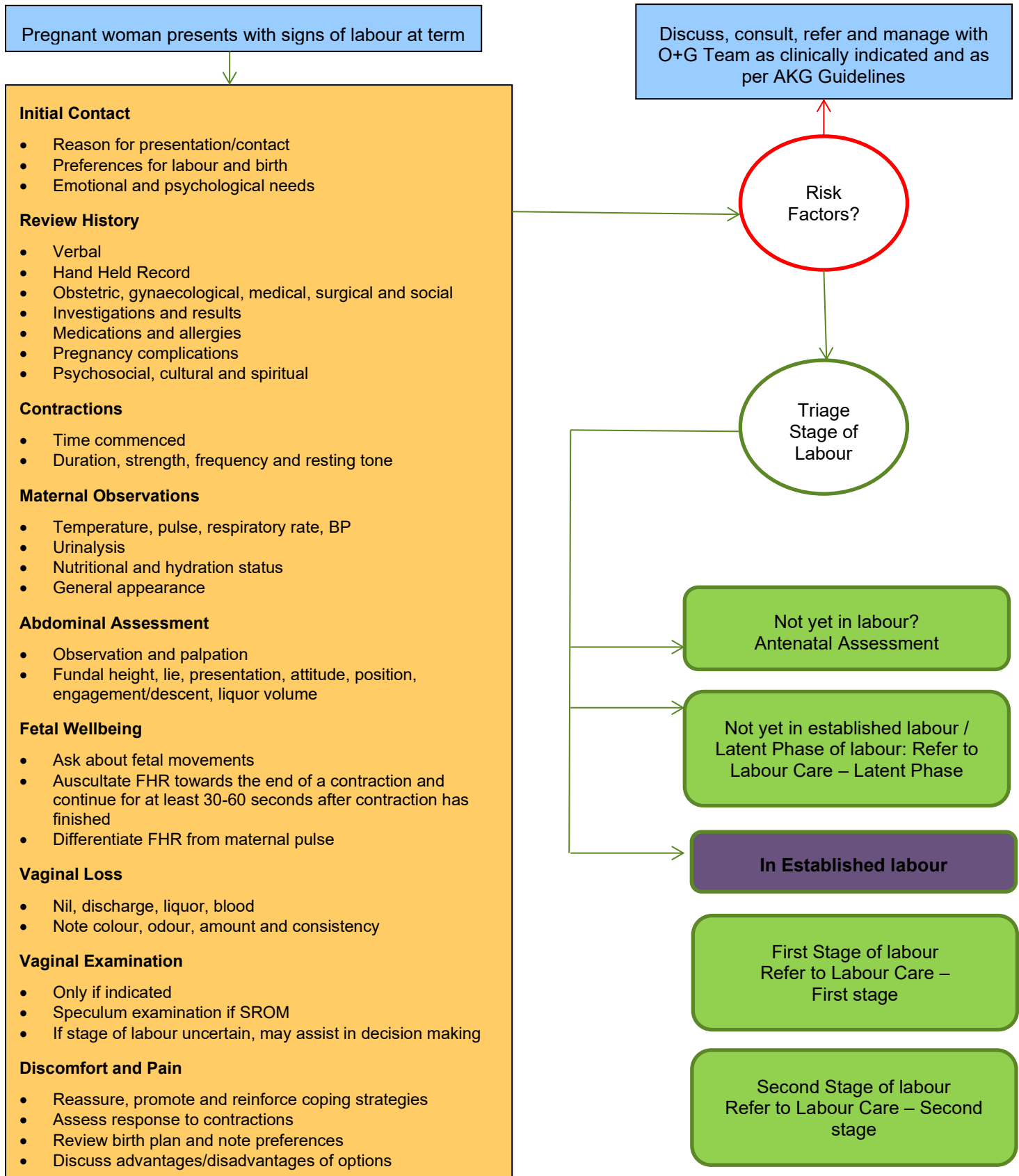
Authorised by	Dr Tania Strickland – Director of Clinical Services
Authorisation date	16/12/2024
Published date	17/12/2024

Appendix A: Initial maternal and fetal assessment

Aspect	Assessment
Initial Contact	• Ascertain reason for presentation or contact. • Assess emotional and psychological needs. • Discuss preferences for labour and birth. • Review history, pregnancy notes and screening results including: ○ Gestational age ○ Past history (medical, obstetric, gynaecological, surgical, social). ○ Medications, allergies. ○ Pregnancy complications. ○ Investigation results (including placental location).
Contractions	• Record time of maternal account of regular, painful contractions. • Assess strength, frequency, duration and resting tone for a 10 minute period.
Maternal Observations	• Temperature, pulse, respiratory rate, blood pressure (BP), and urinalysis. • Assess nutrition and hydration status and general appearance.
Abdomen	• Observation, and palpation including fundal height, fetal lie, presentation, position, engagement / descent and liquor volume.
Fetal Wellbeing	• Ask about fetal movements in the last 24 hours. • Assess FHR: ○ Use a Doppler ultrasound on speaker mode. ○ Auscultate toward the end of a contraction and continue for at least 30–60 seconds after the contraction has finished. ○ Differentiate between the heart beats of the woman and baby. • Routine use of CTG for low risk women is not recommended.
Vaginal Loss	Assess and record vaginal loss: • No loss. ○ Discharge - note colour, odour and consistency. ○ Blood - note colour, volume. ○ Liquor - note colour, volume, odour and consistency.
Vaginal Examination (VE)	• If stage of labour uncertain a VE may assist decision making. • If active labour is suspected offer a VE. • If spontaneous rupture of membranes (SROM) is suspected consider a speculum examination.
Pain and Discomfort	• Assess pain and discomfort both during and between contractions. • Consider need for analgesia.
Contact History	• Review contact history and clinical circumstances with each contact. • Consider the interval since initial contact, and why the woman has re-presented.

Appendix B: Initial assessment

Care is woman centred and includes informed choice, consent, privacy and respectful communication. Contemporaneous documentation is essential

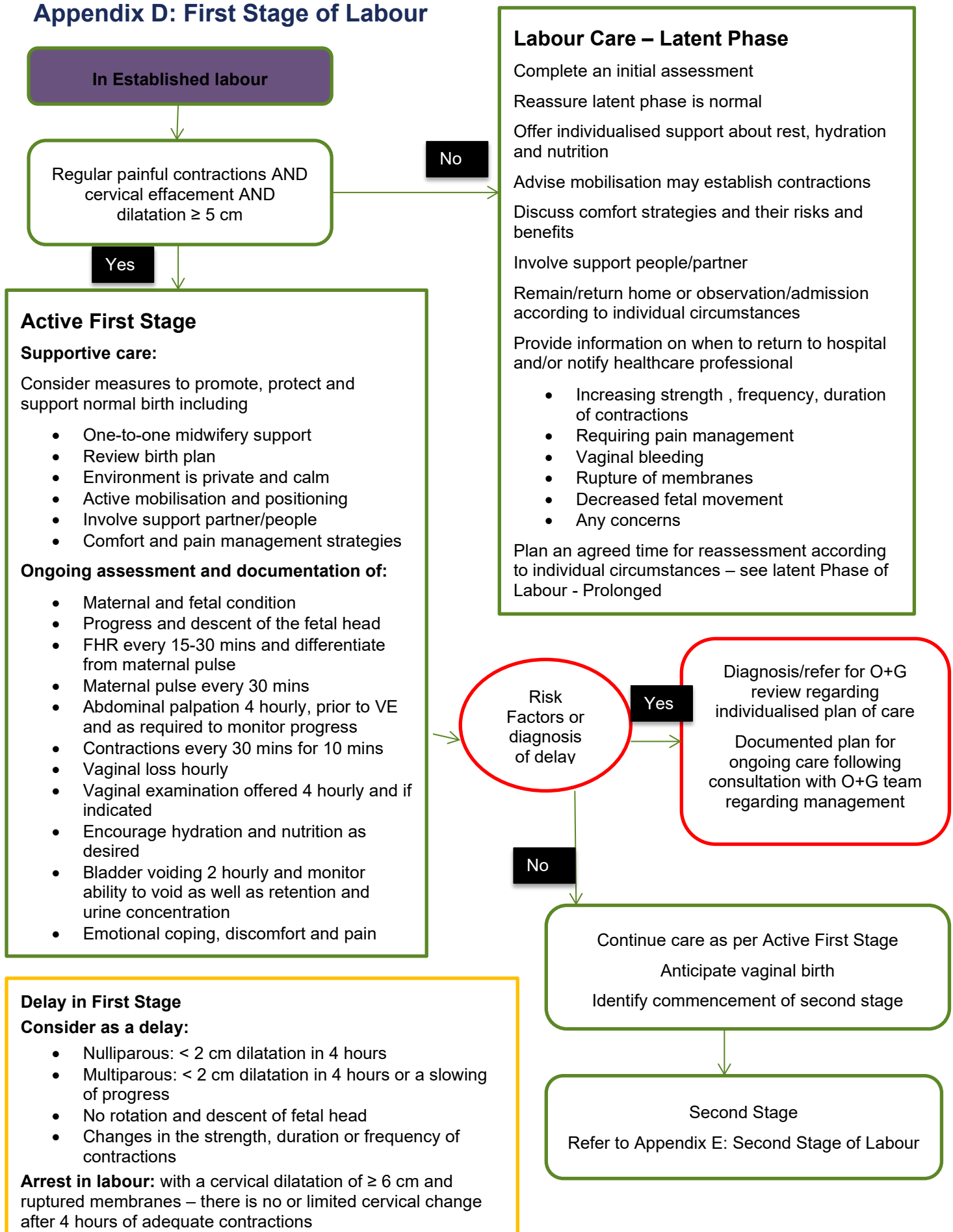


Appendix C: Fetal and maternal observations in first and second stage

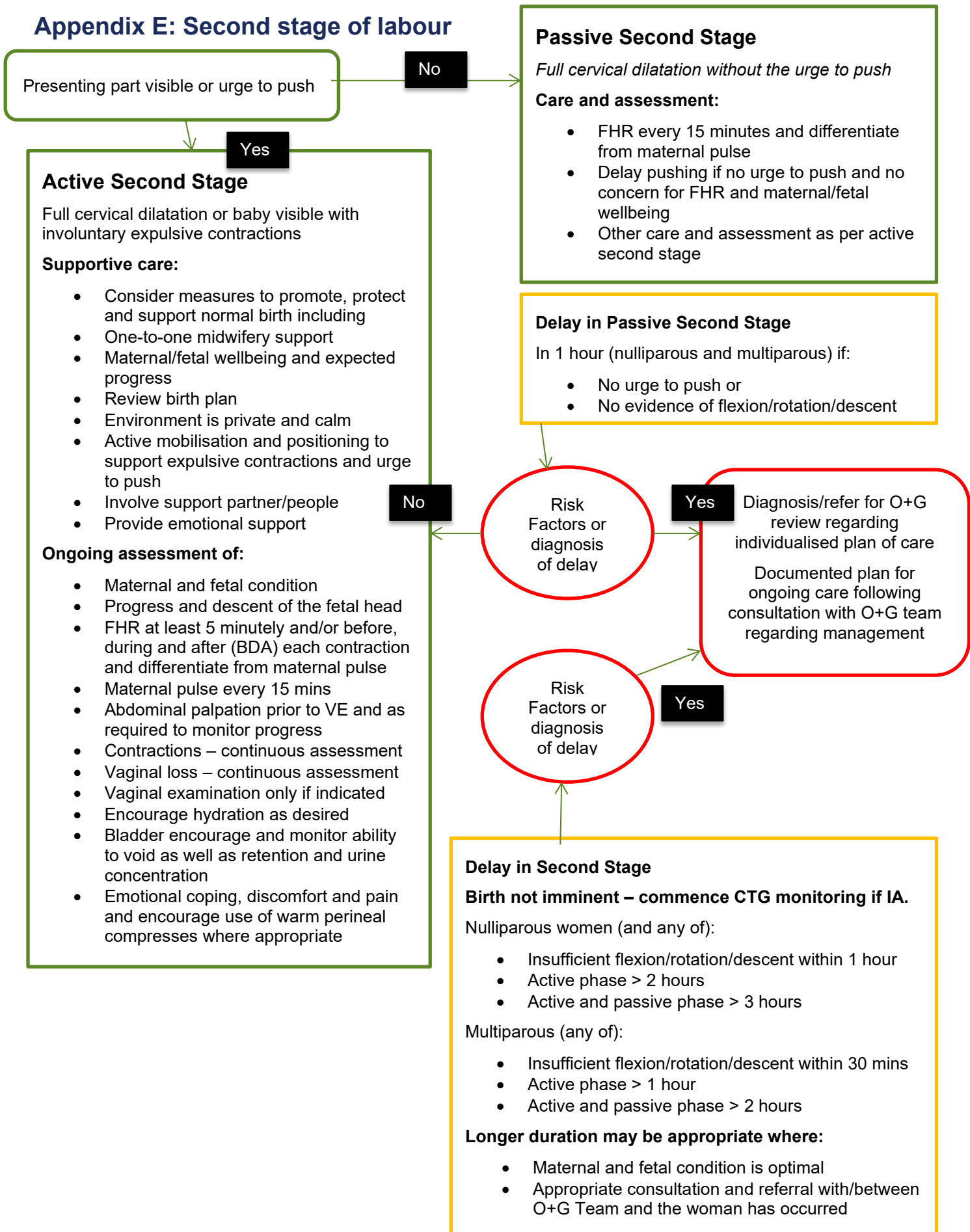
Increase frequency of observations if clinically indicated

Aspect	Latent First Stage (if admitted)	Active First Stage	Second Stage
Fetal Well Being	Every 30 minutes	Every 15-30 mins	FHR at least 5 minutely and/or before, during and after (BDA) each contraction and differentiate from maternal pulse
Temperature	4 hourly 2 hourly if ruptured membranes 1 hourly if febrile	4 hourly 2 hourly if ruptured membranes 1 hourly if febrile	1 hourly
Pulse	4 hourly and always differentiate from FHR	30 minutely and always differentiate from FHR	15 minutely and always differentiate from FHR
Respirations and SaO2	On admission and then 4 hourly Individualised according to analgesic use and medical conditions	4 hourly Individualised according to analgesic use and medical conditions	1 hourly Individualised according to analgesic use and medical conditions
Blood Pressure	4 hourly	2 hourly	1 hourly
Abdominal Palpation	If indicated and as required to monitor progress Prior to a VE	If indicated and as required to monitor progress Prior to a VE	If indicated and as required to monitor progress Prior to a VE
Contractions	Hourly	Every 30 minutes for 10 minutes	Continuous assessment
Vaginal Examination (VE)	If clinically indicated and no contraindications	4 hourly if clinically indicated and more frequently as required to aid decision making	As clinically indicated and as required to aid decision making
Vaginal Loss (Amniotic Fluid) <i>I – Intact</i> <i>C – Clear</i> <i>B – Blood Stained</i> <i>M – Meconium Stained</i>	2 hourly	2 hourly if membranes intact 30 minutely if membranes ruptured	Observe continuously
Bladder Management	Prior to Abdominal Palpation or VE Encourage voiding 2 hourly	Prior to Abdominal Palpation or VE Encourage and monitor 2 hourly voiding IDC if visible and/or palpable bladder IDC in the event of an Epidural	Prior to Abdominal Palpation or VE Monitor and encourage frequent voiding IDC if visible and/or palpable bladder IDC in the event of an Epidural

Appendix D: First Stage of Labour



Appendix E: Second stage of labour



Appendix F: Second stage of labour – perineal care and trauma reduction

Aspect	Consideration
Australian Context and Collaboration	Perineal injury is the most common maternal morbidity associated with vaginal birth • Over 50% of perineal injuries require surgical repair • There has been a reported increased rate of Obstetric Anal Sphincter Injuries (OASIS) • 3rd and 4th degree tears are included on the Australian Commission on Safety and Quality in Healthcare (ACSQHC) Hospital Acquired Complications list • From the 2020–2021 financial year onwards, a financial penalty is applied to public hospital services for each episode of care where a woman sustains a 4th degree tear • Women's Healthcare Australasia (WHA) and more recently the Australian Commission on Safety and Quality in Health Care (ACSQHC) have implemented a bundle of interventions and clinical care standards with the aim of reducing the incidence and rate of OASIS
Intrapartum Experience of Clinician	• The individual clinician providing care at the time of birth is an independent factor that influences risk of perineal trauma • More experienced midwives are reported to have lower rates of severe perineal tears than inexperienced midwives – provide inexperienced student midwives and registered midwives with support as per the individual clinician • Where possible, consider having experienced clinicians manage the birth of women at higher risk for OASIS
Maternal Position	• Greatest incidence of intact perineum in all-fours and kneeling positions • Lowest incidence of OASIS in standing and lateral positions • Greatest incidence and degree of perineal trauma in sitting, squatting and birth-stool positions • Lithotomy and squatting positions associated with increased risk of OASIS • Support women to give birth in whatever position they find comfortable – and accoucheur to maintain good visualisation of perineum • Advise women of the benefits of the all-fours, kneeling positions, lateral and standing positions • Consider increased risk of perineal trauma with birth stool, sitting or squatting and the length of time a woman spends in these positions in second stage
Perineal Warm Compresses in Second Stage	• Reported to Increase second stage comfort and may reduce incidence of OASIS • Not shown to have any effect on incidence of: ○ Intact perineum ○ Perineal trauma requiring suturing ○ Episiotomy rates • Offer warm perineal compresses in second stage of labour and ensure appropriate temperature prior to application and stop at the woman's request
Episiotomy	• Restrict use of mediolateral episiotomy to clinical indications • Provide effective analgesia for the episiotomy except in an emergency situation • Perform episiotomy at the crowning of the fetal head • No consensus on the optimal angle from the midline for mediolateral episiotomy however recommendations are to perform at an angle of 45-60 degrees from the midline

Fetal Head and Perineal Support – Hands ON or Hands OFF (or poised)	• High level evidence does not demonstrate any clear differences between hands on and hands off (or poised) in terms of risk of OASIS. • Episiotomy rates are higher with hands on technique. • For the fetal head - have hands on or hands poised whenever possible ◦ Novice clinicians to have hands on fetal head and peer support whenever possible. • For the perineum - use clinical judgement in determining whether to have hands on or hands off • Continuously watch the perineum and evaluate the risk of injury. • Assist birth of the body of the baby by lateral flexion of the trunk following the curve of Carus • Use minimum amount of force required to achieve birth to reduce risk of perineal injury to woman and traction injury to fetus
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Summary of Intrapartum Perineal Trauma Reduction Strategies:

- Offer in second stage:
 - Perineal warm compresses
- Support woman to give birth in position they find most comfortable
 - Inform of benefits of all-fours, kneeling, lateral and standing positions
 - Avoid prolonged periods in birth stool, sitting, lithotomy and squatting positions
- Closely observe perineum during second stage
- Promote slow and gentle birth of fetal head, shoulders and body
- Communicate clearly, especially in final stages of second stage
- Use hands on or hands poised technique according to clinical situation
- Restrict use of mediolateral episiotomy to clinical indications
- If previous OASIS or multiple risk factors an experienced accoucheur where possible